NAME – AESHA CHAUDHARY
SAP – 590025445
BATCH – 1 (Data Science) BIDHOLI

**CONSTITUTIONAL LAW I**

**BLOG WRITING**

# Right to Die with Dignity: Euthanasia under Article 21 of the Indian Constitution

## Introduction: Does the Right to Life Include a Right to Die with Dignity?

The Constitution of India guarantees every person the **right to life and personal liberty under Article 21**. Judicial interpretation has transformed this short constitutional provision into a broad protection of human dignity, autonomy, privacy and individual choice. Yet, this creates one of the most difficult questions in constitutional law: **if the Constitution protects the right to live with dignity, should dignity also be protected when life is approaching its end?**

**Constitution of India | Legislative Department**

The question becomes particularly difficult when a person is permanently unconscious, suffering from an irreversible medical condition, or dependent upon life-sustaining medical treatment with no realistic prospect of recovery. Modern medicine can prolong biological existence for years, but the ability to prolong life does not automatically answer whether doing so always respects the dignity and wishes of the patient.

Indian constitutional law has gradually responded by drawing an important distinction between a general **"right to die"** and the **"right to die with dignity."** Article 21 does not grant an unrestricted right to end one's life. However, the Supreme Court has recognised that dignity and autonomy may permit withholding or withdrawal of life-sustaining medical treatment in carefully regulated circumstances.

India's approach therefore attempts to balance two important constitutional values: **the State's obligation to protect life and the individual's right to dignity and autonomy at the end of life.**

## Understanding Euthanasia: Active and Passive

Euthanasia broadly concerns decisions that result in the death of a person suffering from an incurable or irreversible condition, particularly where continued medical intervention merely prolongs the process of dying.

A fundamental legal distinction exists between **active and passive euthanasia**.

**Active euthanasia** involves a deliberate positive intervention intended to cause death, such as administering a lethal substance. It remains unlawful in India.

**Passive euthanasia**, on the other hand, involves withholding or withdrawing life-sustaining medical treatment in legally recognised circumstances and allowing the underlying disease or injury to take its natural course.

Importantly, withdrawal of futile medical treatment does not mean abandoning the patient. Medical supervision, pain management, palliative care and appropriate end-of-life care remain essential.

This distinction forms the foundation of India's legal framework governing end-of-life decisions.

### Article 21: From the Right to Life to Dignity at Death

Article 21 states:

"No person shall be deprived of his life or personal liberty except according to procedure established by law."

Although the provision expressly speaks of "life," the Supreme Court has repeatedly interpreted life as something more meaningful than mere biological survival.

The euthanasia debate therefore asks an important constitutional question: **Can compelling a person to undergo unwanted or medically futile life-sustaining treatment violate dignity, autonomy and bodily integrity?**

The Supreme Court's answer has developed gradually through several landmark cases.

### *P. Rathinam v. Union of India* (1994): The Initial "Right to Die"

In ***P. Rathinam v. Union of India***, the Supreme Court examined the constitutional validity of Section 309 of the Indian Penal Code, which criminalised an attempt to commit suicide.

The Court controversially concluded that the right to life under Article 21 also contained a **right not to live**. Section 309 was consequently held unconstitutional.

The reasoning, however, created a major constitutional difficulty. Could a fundamental right specifically intended to protect human life simultaneously provide a general constitutional entitlement to extinguish that life?

The issue soon returned to the Supreme Court. P.Rathinam vs Union Of India on 26 April, 1994

### *Gian Kaur v. State of Punjab* (1996): Drawing the Essential Distinction

A Constitution Bench reconsidered the issue in ***Gian Kaur v. State of Punjab***, (1996) 2 SCC 648.

The Court overruled *P. Rathinam* and held that the **right to life under Article 21 does not include a general right to die**. Suicide involves the premature termination of life and cannot simply be regarded as the negative dimension of the right to life.

However, *Gian Kaur* made another observation that ultimately became more important to the development of euthanasia law.

The Court distinguished an unrestricted right to die from a **right to die with dignity during the process of natural death**. Human dignity, it reasoned, does not disappear when a person reaches the final stages of life.

Thus, *Gian Kaur* rejected a constitutional right to suicide while leaving open the possibility that Article 21 could protect dignity during an irreversible process of dying.

***Smt. Gian Kaur vs The State Of Punjab on 21 March, 1996***

### *Aruna Ramachandra Shanbaug v. Union of India* (2011): Passive Euthanasia Recognised

The issue became more concrete in ***Aruna Ramachandra Shanbaug v. Union of India***, (2011) 4 SCC 454.

Aruna Shanbaug, a nurse at Mumbai's KEM Hospital, suffered catastrophic brain damage following a brutal assault in 1973 and remained in a severely impaired condition for decades. A petition was eventually filed seeking permission for withdrawal of the support sustaining her life.

The Supreme Court did not permit withdrawal in Aruna's particular circumstances. Nevertheless, the judgment was a turning point because the Court recognised the legal possibility of **passive euthanasia subject to strict safeguards**.

The decision also highlighted the importance of patient autonomy. A competent adult ordinarily possesses the right to decide what medical treatment may be performed on his or her body, including the right to refuse treatment.

The harder question remained: **Who should decide when the patient has permanently lost the ability to communicate?**

### *Common Cause v. Union of India* (2018): Right to Die with Dignity

The constitutional position became substantially clearer in ***Common Cause (A Registered Society) v. Union of India***, (2018) 5 SCC 1.

A Constitution Bench of the Supreme Court held that the **right to die with dignity forms part of Article 21** and recognised the legality of withholding or withdrawing life-sustaining treatment in appropriate circumstances.

The Court also recognised **Advance Medical Directives**, commonly called **living wills**.

A living will allows a competent adult to record preferences regarding medical treatment in advance, particularly for a future situation in which that person becomes incapable of communicating or making medical decisions.

The judgment connected end-of-life decision-making with the constitutional principles of **dignity, bodily integrity, privacy and individual autonomy**.

However, the Court did not create an unrestricted licence to end life. It established procedural safeguards intended to prevent coercion, abuse and arbitrary medical decisions.

In **2023**, the Supreme Court modified aspects of the procedure laid down in *Common Cause* to address practical difficulties surrounding Advance Medical Directives and end-of-life decision-making. (*Common Cause v. Union of India*, M.A. No. 1699/2019 in W.P.(C) No. 215/2005, order dated 24 January 2023).

The constitutional principle is therefore better understood as **regulated autonomy rather than absolute autonomy** Common Cause (A Regd. Society) v. Union of India (2018)

***Harish Rana v. Union of India* (2026): A New Chapter**

India's end-of-life jurisprudence entered another important stage in ***Harish Rana v. Union of India***, **2026 INSC 222**, decided by the Supreme Court on **11 March 2026**.

Harish Rana had suffered a severe brain injury following a fall in 2013. He remained in a permanent vegetative state for more than a decade and was sustained through **Clinically Assisted Nutrition and Hydration (CANH)** administered through a feeding tube.

2025 7 1501 69246 Judgement 11-Mar-2026 | PDF | Health Care | Clinical Medicine

The Supreme Court concluded that CANH administered through such medical intervention could constitute **medical treatment** capable of withdrawal within India's passive-euthanasia framework. It considered the irreversible nature of Harish's condition, medical assessments and the absence of therapeutic benefit from continuing treatment. (*Harish Rana v. Union of India*, 2026 INSC 222).

The Court also emphasised the **"best interests of the patient"** principle when the patient lacks decision-making capacity. This assessment is not confined to whether the body can technically be kept alive. It involves medical prognosis, possibility of recovery, therapeutic benefit, the patient's dignity, expert opinion and consultation with those caring for the patient.

The judgment is especially important because the Court made clear that **withdrawal of life-sustaining treatment cannot mean abandonment of the patient**. The transition must be towards appropriate palliative and end-of-life care so that dignity is maintained throughout the dying process.

The Court also streamlined aspects of the *Common Cause* framework and highlighted the continuing need for **comprehensive legislation governing end-of-life care in India**.

**Suicide and Euthanasia Are Not the Same**

An important misconception must be avoided: **euthanasia and suicide are not legally identical concepts**.

Section 115 of the **Mental Healthcare Act, 2017** creates a presumption, unless proved otherwise, that a person attempting suicide was under severe stress and requires a care-oriented response. The Act places responsibility on the appropriate government to provide care, treatment and rehabilitation aimed at reducing the risk of recurrence.

India's criminal-law framework also changed after the **Bharatiya Nyaya Sanhita, 2023 (BNS)** came into force on **1 July 2024**. The BNS does not reproduce the former Section 309 IPC as a general offence in the same form. Instead, **Section 226** specifically addresses an attempt to commit suicide with the intent to compel or restrain a public servant from exercising lawful power.

These developments do not establish a constitutional right to suicide.

Passive euthanasia rests on a different legal foundation: **medical autonomy, refusal or withdrawal of treatment, dignity and carefully controlled end-of-life decision-making**.

**The Constitutional Debate: Autonomy versus Sanctity of Life**

The strongest argument supporting passive euthanasia is **individual autonomy**.

Medical technology can maintain biological functions for extraordinary periods. Yet the constitutional value of life cannot necessarily be reduced to the continued functioning of the human body. When treatment has become medically futile and the underlying condition is irreversible, forcing unwanted intervention may conflict with bodily integrity and human dignity.

There is also an important moral and legal difference between **causing death and allowing natural death**. Passive euthanasia does not authorise doctors to deliberately kill patients. Instead, in narrowly defined circumstances, it permits futile or unwanted medical intervention to be withdrawn so that the underlying condition takes its natural course.

However, serious arguments exist against unrestricted euthanasia.

India faces substantial economic and social inequalities. **Elderly persons, persons with disabilities and financially dependent patients may be vulnerable to family pressure, neglect or decisions influenced by the financial burden of prolonged treatment.** What appears to be an exercise of autonomy could, without adequate safeguards, become coercion disguised as consent.

There is another problem: medical predictions are not infallible. Diagnoses and prognoses may change, while a decision resulting in death cannot be reversed.

Therefore, safeguards such as independent medical assessment, informed decision-making, documentation, consultation, review mechanisms and the patient's best interests should not simply be dismissed as bureaucratic obstacles. They are necessary to ensure that the **right to die with dignity never becomes pressure upon vulnerable people to die**.

At the same time, safeguards should not become so complicated that a constitutional right becomes practically impossible to exercise. The challenge is to find the correct balance between **accessibility and protection against abuse**.

### The Way Forward: India Needs Comprehensive Legislation

India's constitutional jurisprudence has developed considerably, but the framework continues to depend heavily on judicial decisions.

This is not ideal for patients, doctors or families facing emotionally difficult end-of-life decisions.

Parliament should enact comprehensive **end-of-life care legislation** governing Advance Medical Directives, withdrawal of futile treatment, medical-board procedures, protection of vulnerable patients, duties and legal protection of healthcare professionals, family participation and access to palliative care.

The legislation should also create simple and accessible procedures so that living wills do not remain rights existing mainly on paper.

A statutory framework could provide greater certainty while ensuring that individual autonomy does not become a pathway for coercion, neglect or economically motivated decisions.

### Conclusion: Dignity Must Continue Until the End

India's euthanasia jurisprudence demonstrates how the meaning of Article 21 has evolved.

From *P. Rathinam* to *Gian Kaur*, *Aruna Shanbaug*, *Common Cause* and now *Harish Rana*, the Supreme Court has gradually developed a careful constitutional distinction:

**Article 21 does not guarantee a general right to die. It does, however, protect the right to live—and, in legally recognised end-of-life circumstances, to die—with dignity.**

The distinction is essential. The law should neither treat human life as disposable nor insist that biological existence must be prolonged at every cost, irrespective of medical futility, patient autonomy and dignity.

Ultimately, the constitutional question is not merely whether the State should protect life. It unquestionably should. The deeper question is **what protecting human dignity means when medical science can prolong life but cannot restore the person to a meaningful state of recovery**.

A compassionate constitutional democracy must therefore protect life while respecting autonomy and simultaneously protecting vulnerable individuals from coercion.

India's developing framework of passive euthanasia attempts to achieve precisely this balance: **protecting life without forgetting the dignity of the person living it.**

### References and Authoritative Sources

1. **Constitution of India**, Article 21 – Protection of Life and Personal Liberty.
2. ***P. Rathinam v. Union of India***, (1994) 3 SCC 394.

   P.Rathinam vs Union Of India on 26 April, 1994
3. ***Gian Kaur v. State of Punjab***, (1996) 2 SCC 648.

   Smt. Gian Kaur vs The State Of Punjab on 21 March, 1996
4. ***Aruna Ramachandra Shanbaug v. Union of India***, (2011) 4 SCC 454.
5. ***Common Cause (A Registered Society) v. Union of India***, (2018) 5 SCC 1.

   Common Cause (A Regd. Society) v. Union of India (2018)
6. ***Common Cause v. Union of India***, M.A. No. 1699/2019 in W.P.(C) No. 215/2005, order dated 24 January 2023 – modification of Advance Medical Directive guidelines.
7. ***Harish Rana v. Union of India***, 2026 INSC 222, Supreme Court of India, judgment dated 11 March 2026.

   2025 7 1501 69246 Judgement 11-Mar-2026 | PDF | Health Care | Clinical Medicine
8. **Mental Healthcare Act, 2017**, Section 115 – India Code, Ministry of Health and Family Welfare.
9. **Bharatiya Nyaya Sanhita, 2023**, Section 226 – India Code, Ministry of Home Affairs.

   The Bharatiya Nyaya Sanhita, 2023
10. **Supreme Court of India**, "Landmark Judgment Summaries – *Harish Rana v. Union of India*," 2026.